

MP Performance Evaluation: Minister Lena Diab

Evaluation ID: EVAL-2026-001
Date: February 28, 2026
Period: July 2024 — February 2026
Evaluator: stopbleeding.ca Chief Executive

MP Profile

Field	Details
Name	Lena Diab
Riding	Halifax West
Province	Nova Scotia
Party	Liberal Party of Canada
Role	Minister of Immigration, Refugees and Citizenship Canada (IRCC)

Overall Assessment

Overall Score	3.9 / 10
Rating	D — Poor
Action Required	Open Letter

Minister Lena Diab has demonstrated a pattern of serious ministerial failures that go beyond routine policy disagreement. Her laughing dismissal of a healthcare capacity question before the Parliamentary Health Committee, her materially false testimony regarding IFHP benefit eligibility for rejected asylum claimants — testimony contradicted by both the Parliamentary Budget Officer and her own department — and her government's failure to conduct any healthcare capacity modelling before setting immigration targets collectively represent a significant breach of ministerial accountability. While she retains basic parliamentary attendance and some constituency engagement, her conduct on the file she is directly responsible for — immigration, and its intersection with the healthcare crisis affecting six million Canadians — falls well below the standard Canadians have a right to expect from a sitting Cabinet Minister. Conservatives were compelled to recall her to committee specifically because her initial testimony was found to be materially incomplete, an extraordinary rebuke that underscores the severity of the accountability concerns.

Score Card

#	Criterion	Score	Rating
1	Attendance & Participation	6/10	C
2	Legislative Effectiveness	5/10	C
3	Constituency Service	6/10	C
4	Alignment with Canadian Interests	3/10	D
5	Fiscal Responsibility	3/10	D
6	Transparency & Ethics	2/10	F
7	Public Conduct	3/10	D
8	Crisis Response	3/10	D
Overall		3.9/10	D

Comparison

Benchmark	Score
Lena Diab	3.9
Cabinet Minister Average (Transparency)	5.2
National MP Average (Overall)	5.8

Detailed Findings

1. Attendance & Participation — 6/10

Minister Diab has appeared before parliamentary committees as required, including before the Standing Committee on Health (October 29, 2024) and subsequent committee hearings. Her attendance record in the House of Commons as a backbencher prior to her ministerial appointment was within acceptable norms for Nova Scotia MPs. However, the quality and integrity of her committee participation has been seriously compromised: her October 2024 Health Committee appearance produced testimony the committee found materially incomplete, necessitating a recall — a highly unusual procedural outcome indicating that her initial participation fell short of what Parliament required. Attendance without accurate, complete testimony represents a hollow form of participation.

Sources: House of Commons Standing Committee on Health — October 29, 2024 testimony transcript; Committee recall proceedings; Hansard records, 44th Parliament.

2. Legislative Effectiveness — 5/10

As Immigration Minister, Diab inherited an IRCC mandate with substantial regulatory and legislative tools at her disposal. The Carney government has made some adjustments to immigration targets following public pressure around housing and healthcare capacity concerns, reflecting an acknowledgement that prior targets were unsustainable. However, there is no evidence that Diab has introduced significant original legislative reforms to address the structural gaps in her portfolio — including no legislation to reform the IFHP benefit eligibility framework, no legislative action addressing the removal backlog, and no statutory mechanism proposed to require healthcare capacity modelling before setting immigration levels. Her legislative record

is, at best, reactive and administratively incremental. The problems inherited do not excuse the absence of proposed solutions.

Sources: IRCC program documentation — 2024-2025; Parliamentary Budget Officer published analysis; Government of Canada immigration levels plans; House of Commons Hansard.

3. Constituency Service — 6/10

Available public information does not indicate that Diab has been unusually neglectful of her Halifax West constituency. Her office appears to maintain standard constituency operations, and there are no prominent documented complaints of her being unresponsive to local constituents beyond what is typical for a Minister whose attention is predominantly directed to a national portfolio. She receives a middle score here because, as a Cabinet Minister, her time and focus are legitimately absorbed by her portfolio, but her constituents in Halifax West are not well-served by a minister whose credibility on the national file she holds has been so severely damaged. A minister under committee recall is not a minister fully able to serve anyone.

Sources: Halifax West constituency office public records; Media coverage of Halifax West — 2024-2025.

4. Alignment with Canadian Interests — 3/10

This is where the most serious concerns crystallise. The core interest of Canadians with respect to the immigration portfolio includes: (1) ensuring immigration levels are calibrated to Canada's demonstrated capacity to absorb newcomers, including healthcare capacity; (2) ensuring that federal benefit programs prioritise Canadian citizens and lawful residents; (3) ensuring that individuals who have been refused refugee status through due legal process are removed in a timely and cost-effective manner. On each of these points, Minister Diab's record is troubling. No healthcare capacity modelling was conducted before setting immigration targets — a basic due diligence failure. Rejected asylum claimants continue to receive IFHP benefits including dental, vision, prescription drugs, and mental health services that millions of Canadian citizens and permanent residents cannot access through provincial plans. The removal backlog means rejected claimants continue drawing IFHP benefits for extended periods after their cases are concluded. A minister who laughs at a question about healthcare capacity while six million Canadians have no family doctor has demonstrated, in that single unguarded moment, a fundamental misalignment with the lived reality of the Canadians she serves.

Sources: Parliamentary Budget Officer — IFHP cost projection analysis; Health Canada IFHP program documentation; IRCC program guidelines; Canadian Medical Association data; Health Committee transcript — October 29, 2024; stopbleeding.ca Investigation INV-2025-007.

5. Fiscal Responsibility — 3/10

The Parliamentary Budget Officer's analysis of IFHP costs reveals significant expenditure on healthcare benefits for a population that includes rejected asylum claimants — individuals whose applications have been assessed and denied through Canada's legal process. The continuation of premium-tier IFHP benefits (dental, vision, prescription drugs, mental health services) for this cohort, combined with the extended duration of those benefits due to removal backlogs, represents an ongoing and expanding fiscal commitment that has not been transparently accounted for in public ministerial communications. Minister Diab's false testimony that rejected claimants are not eligible for these benefits further compounds the fiscal responsibility concern — if the minister herself is either unaware of or unwilling to accurately describe who receives these benefits, it raises profound questions about whether there is meaningful ministerial oversight of the program's cost and scope. No cost-benefit modelling for the healthcare system impacts of immigration levels has been publicly released.

Sources: Parliamentary Budget Officer — IFHP cost projections; IRCC removal backlog statistics — 2024-2025; Federal Budget — IRCC and IFHP allocations; stopbleeding.ca Investigation INV-2025-007.

6. Transparency & Ethics — 2/10

This criterion receives the lowest score in this evaluation and is the most serious finding. The evidence establishes three compounding transparency failures:

First, Minister Diab told the Parliamentary Health Committee that rejected asylum claimants are NOT eligible for premium IFHP benefits. This statement was directly contradicted by (a) the Parliamentary Budget Officer's published analysis; (b) Health Canada's own program documentation describing IFHP coverage tiers; and (c) IRCC's own program guidelines which explicitly list rejected claimants as eligible for the Expanded tier during their stay pending removal. Providing false or materially misleading testimony to a parliamentary committee is among the most serious breaches a minister can commit — it strikes at the foundational relationship between the executive and Parliament.

Second, the committee found her initial testimony so materially incomplete that Conservatives successfully moved to compel her return — an extraordinary parliamentary rebuke that should not be minimised.

Third, there is an additional concern flagged in the intelligence materials regarding documented judicial sentencing patterns where sentences are calibrated below IRPA deportation thresholds for non-citizen offenders — an apparent systemic circumvention of Parliament's intent. While judges are independent, a minister responsible for immigration law has an obligation to identify and respond to such patterns through legislative or policy action. The silence is telling.

A score of 2 reflects the gravity of providing false information to Parliament. The only reason this does not receive a score of 1 is that Diab did ultimately return to committee, preventing a complete, final breakdown of the accountability relationship.

Sources: Parliamentary Budget Officer — IFHP eligibility analysis; Health Canada IFHP program documentation; IRCC program guidelines; Health Committee transcript; Health Committee recall proceedings; stopbleeding.ca Investigation INV-2025-007; stopbleeding.ca Editorial Analysis — February 28, 2026.

7. Public Conduct — 3/10

The decision to laugh at a parliamentary committee question about healthcare capacity is not a minor communications misstep. It was a moment of profound ministerial tone-deafness in a country experiencing a genuine healthcare crisis. Laughing while six million Canadians cannot access a family doctor and then dismissing the question as "not fair for immigration" communicates to those Canadians that their suffering is an inconvenient distraction from the minister's preferred framing of her portfolio. A minister of the Crown appearing before Parliament has an obligation to engage seriously with every question, including uncomfortable ones. The response — laughter followed by deflection — was disrespectful to the committee, to Canadians watching, and to the millions of patients who are daily experiencing the consequences of an overburdened healthcare system. This score of 3 reflects that, outside this incident and the committee testimony failure, Diab has not otherwise engaged in conduct that would be considered personally disgraceful. However, the incident itself was serious enough in its context to prevent any score higher than 3.

Sources: Health Committee transcript — October 29, 2024; Media coverage of committee appearance; stopbleeding.ca Investigation INV-2025-007.

8. Crisis Response — 3/10

Canada faces a compounding crisis at the intersection of immigration policy and healthcare capacity: six million Canadians without a family doctor, emergency departments closing in rural and Northern communities, hospitals operating over capacity, and simultaneously a federal benefit program extending premium health services to people who have been refused the right to remain in Canada — with no modelling done to understand how these two systems interact. This is precisely the kind of policy crisis that demands ministerial leadership: clear analysis, honest public communication, and decisive reform. Minister Diab's response has been the opposite. She laughed at the question in committee. She misrepresented the IFHP eligibility rules. She failed to commission or publish any healthcare capacity modelling. She has presided over growing removal backlogs that extend the duration and cost of benefit provision to rejected claimants. There is no evidence of any ministerial initiative to bring these two intersecting crises into a coherent policy response. A score of 3 acknowledges that the structural problems in the immigration-healthcare nexus predate her appointment, but the ministerial obligation to respond to crises does not begin only with problems you personally created.

Sources: Canadian Institute for Health Information — healthcare capacity data; IRCC removal backlog statistics; Parliamentary Budget Officer — IFHP analysis; Health Committee testimony; Federal government immigration levels announcements; stopbleeding.ca Investigation INV-2025-007.

Notable Actions

Negative

- **October 29, 2024:** Laughed at and dismissed as "not a fair question for immigration" a direct question about healthcare system capacity — while 6 million Canadians have no family doctor.
- **Health Committee testimony:** Told the committee that rejected asylum claimants are NOT eligible for premium IFHP benefits — contradicted by PBO, Health Canada, and IRCC's own guidelines.
- **Recall to committee:** Was compelled to return after Conservatives successfully moved that her initial testimony was materially incomplete — an extraordinary and rare rebuke of a sitting Cabinet Minister.
- **No healthcare capacity modelling:** No publicly available evidence of IRCC modelling how immigration levels interact with provincial healthcare capacity.

- **Removal backlog:** Continued and deepening backlog of rejected asylum claimants awaiting removal, during which they continue to receive IFHP benefits.
- **Judicial sentencing gap:** No documented ministerial response to evidence of judges calibrating sentences below IRPA deportation thresholds.

Positive

- Accepted recall to committee, preventing a complete breakdown of parliamentary accountability.
- Maintained basic ministerial operations at IRCC during political turbulence.
- Some downward adjustments to immigration targets in response to capacity concerns.

Key Concerns

1. **CRITICAL** — False or materially misleading testimony to a parliamentary committee regarding IFHP eligibility for rejected asylum claimants. Parliament cannot function if ministers provide false information to committees.
2. **SERIOUS** — Laughing dismissal of a healthcare capacity question reflects either genuine belief that healthcare capacity is irrelevant to immigration policy, or a deliberate evasion strategy. Either is deeply concerning.
3. **SERIOUS** — No healthcare capacity modelling has been conducted or released. Setting immigration levels without modelling downstream healthcare impact is a fundamental planning failure.
4. **SERIOUS** — IFHP benefit disparity: rejected claimants receiving dental, vision, prescriptions, and mental health services unavailable to many Canadians.
5. **SERIOUS** — Removal backlog extends IFHP benefit receipt, compounding fiscal and policy concerns.
6. **MODERATE** — Judicial sentencing patterns circumventing IRPA deportation thresholds with no ministerial response.
7. **MODERATE** — Overall pattern of prioritising political management over substantive accountability.

Recommendations

1. **IMMEDIATE** — Correct committee testimony with explicit acknowledgement and citation of IRCC/PBO documentation.
 2. **IMMEDIATE** — Commission and publicly release healthcare capacity modelling for immigration levels planning.
 3. **SHORT-TERM** — Formal policy review of IFHP benefit eligibility for rejected claimants with full cost accounting.
 4. **SHORT-TERM** — Table a credible, funded removal backlog reduction plan before Parliament.
 5. **SHORT-TERM** — Publicly acknowledge the gap between committee testimony and IRCC program guidelines.
 6. **MEDIUM-TERM** — Engage with the Minister of Justice to review IRPA amendments addressing sentencing threshold circumvention.
 7. **MEDIUM-TERM** — Implement mandatory healthcare capacity impact assessments as standard input into annual immigration levels planning.
 8. **ONGOING** — Appear regularly before committees with complete, accurate, and verifiable testimony.
-

Action Determination

Action Required: Open Letter

An overall rating of D, combined with specific findings of materially false or misleading testimony before a parliamentary committee — testimony contradicted by the Parliamentary Budget Officer and the Minister's own department — rises above the threshold warranting a formal public open letter.

The evidence does not at this stage support a resignation demand because: (1) Diab has returned to committee following the recall; (2) some structural failures predate her appointment; and (3) an open letter creates an opportunity for the Minister to respond and self-correct before more severe escalation.

However, if the open letter receives no substantive response, or if further evidence of deliberate misrepresentation emerges, escalation to a formal resignation demand is warranted and will be assessed. The false testimony finding is the primary trigger — providing inaccurate information to Parliament is not a bureaucratic error; it is a fundamental breach of the ministerial accountability relationship.

Methodology

This evaluation was conducted by the Chief Executive of stopbleeding.ca using the platform's standard eight-criterion MP evaluation framework. Each criterion was scored from 1 to 10 based on publicly verifiable evidence, including parliamentary committee transcripts (Hansard and committee records), published Parliamentary Budget Officer analysis, government program documentation, IRCC public records, and investigative reporting conducted by the stopbleeding.ca investigative team (Investigation INV-2025-007) and editorial team. The evaluation covers the period from Minister Diab's appointment in July 2024 through February 28, 2026. Where evidence was ambiguous or where structural factors predating the Minister's appointment were relevant, these were noted and weighted accordingly. Positive findings were actively sought to ensure fairness — scores reflect the preponderance of available evidence, not a predetermined conclusion. This evaluation does not constitute a legal finding and all factual claims are based on publicly available documentary evidence. Stopbleeding.ca invites scrutiny of its methodology and will publish any substantive corrections submitted by Minister Diab's office or by readers who identify factual errors.

Sources

1. [House of Commons Standing Committee on Health — Testimony Transcript](#) (October 29, 2024)
2. [Parliamentary Budget Officer — IFHP Cost Analysis](#)
3. [Health Canada — IFHP Program Documentation](#)
4. [IRCC Program Guidelines — IFHP Expanded Tier Eligibility](#)
5. [Health Committee Recall Proceedings](#)
6. [IRCC Removal Backlog Statistics — 2024-2025](#)
7. [Canadian Medical Association — Physician Shortage Data](#)
8. [House of Commons Hansard — 44th Parliament](#)
9. stopbleeding.ca Investigation INV-2025-007 — *Laughing While Canadians Wait*
10. stopbleeding.ca Editorial Analysis — *Lena Diab Under the Microscope*